

A Master long-case proforma and the 8–10 minute presentation

Use with every long-case card · Long case: 80 marks · 20 + 10 minutes

The department's shorthand

The order and wording on this card come from your department's own case sheets; each subject card adds only what is special to its case. Learn the written shorthand too: last year two long cases were full written case sheets.

Written	Meaning	Written	Meaning
c/o	complaints of	EOM	extraocular movements
h/o, No h/o	history of, no history of	Tn by GAT	tension (intraocular pressure) by Goldmann applanation tonometer
k/c/o	known case of	CCT	central corneal thickness
Ĉ	with	VH Gr IV (ND)	Van Herick grade 4, normal depth
⊕	present, positive	KPs	keratic precipitates
Ⓝ	normal	CPN (or NCP)	iris colour and pattern normal
s/o	suggestive of	RTL (3 mm)	pupil round, reacting to light, 3 mm
RE, LE, BE, B/L	right eye, left eye, both eyes, bilateral	RAPD	relative afferent pupillary defect
DM, SHTN, TB	diabetes mellitus, systemic hypertension, tuberculosis	PCIOL	posterior chamber intraocular lens
PR, BP, RR	pulse rate, blood pressure, respiratory rate	IMC (NS-3)	immature cataract, nuclear sclerosis grade 3
CVS, RS, P/A, CNS	cardiovascular system, respiratory system, per abdomen, central nervous system	PxF in ALC	pseudoexfoliation on the anterior lens capsule
S1 S2	first and second heart sounds	SS, TM, PAS	scleral spur, trabecular meshwork, peripheral anterior synechiae
NVBS	normal vesicular breath sounds	CDR, NRR, PPA	cup-disc ratio, neuroretinal rim, peripapillary atrophy
NFND	no focal neurological deficit	FR ⊕	foveal reflex present
E/D, BD	eye drops, twice daily	PH, DSph	pinhole, dioptre sphere
NIGNIP	no improvement with glasses or pinhole	6/12P	6/12, partial

Two traps: **VH** on the anterior chamber means Van Herick, but on the department's fundus drawings it means vitreous haemorrhage, so write it out. **PVD** in the seniors' checklist means peripheral vascular disease.

Particulars and chief complaints

Particulars: name (Mr X), age, sex, occupation, place; the medical record number goes on the written sheet.

Write the opening line like this: "Mr X, 35-year-old male, labourer by occupation, came with c/o pain, redness, watering and defective vision (LE) for the past 10 days." For a review patient: "Mr X, 70-year-old male, has come for regular follow-up."

- The eye in brackets after the complaints; the duration as "for the past ...".
- Several complaints: in chronological order, each with its own eye and duration.
- The patient's complaint ("prominence of both eyes", "drooping of the left upper lid"), never your diagnosis.

History of presenting illness

Template: "Patient was apparently normal ____ (days / months / years) back, then he noticed ____ in the ____ eye, which was:"

- insidious / sudden / gradual in onset;
- progressive / stationary in nature;
- associated / not associated with ____ (pain, redness, watering, photophobia, headache).

"He visited an ophthalmologist at his place, was found to have ____ and was started on _____. Then he came to our institution for further management." Then the positives ("H/o ____ ⊕"), then the negatives ("No h/o ____"); the department starts each line.

Elaborate each complaint:

Complaint	Ask
Defective vision	Eye, onset, progression; distance or near; pain; night blindness; change of glasses; coloured haloes
Pain	Onset, site, character, radiation; aggravating and relieving factors; eye movement; time of day

Complaint	Ask
Redness, watering	Onset; pain, photophobia, discharge; foreign-body sensation; lid swelling
Double vision	Goes when either eye is closed?; horizontal or vertical; worse in which gaze; diurnal variation
Prominence of the eye	One or both; onset, progression; intermittent?; change with posture or straining; swishing sound
Drooping lid, squint	Congenital or acquired; progression; constant or intermittent; diurnal variation; glasses, patching

Negative history

- Each “No h/o ...” line rules out a named differential. Examiners ask “Why did you ask about ...?”; each card gives the reasons.
- The department’s glaucoma screen: No h/o headache with vomiting and pain · coloured haloes · redness, watering, pain · trauma to the eye · frequent change of glasses.

Past, personal, family and treatment history

- Past** (department checklist): “K/c/o DM / SHTN / TB / asthma / epilepsy for ___ years, on treatment ___” · similar complaints in the past · ocular surgery or trauma: eye, what, when, where.
- Seniors’ drug and systemic checklist, every case:** sulpha allergy · other drug allergy · medical history · head injury · asthma · “CVD” (heart disease or stroke: ask both) · migraine · Raynaud’s phenomenon · peripheral vascular disease · steroid use (type, duration).
- Personal:** “Takes mixed diet. Smoker / alcoholic for ___ years. Normal bowel and bladder habits.”
- Family:** “No h/o similar illness in the family members”; ask about glaucoma, night blindness and consanguinity.
- Treatment:** drug, strength, frequency, eye, duration: “E/D timolol 0.5% BD (BE) for the past ___ years”. Or: “Treated at a private clinic nearby with two eye drops, then referred to our institute.”

General and systemic examination

- General:** “Patient is conscious, oriented, afebrile; moderately built and nourished; not anaemic, not icteric; no cyanosis, clubbing, generalised lymphadenopathy or pedal oedema.”
- Vitals:** “PR ___/min, regular in rhythm, normal in volume · BP ___/___ mm Hg, measured in the left arm in sitting posture · RR ___/min.”
- Systemic:** “CVS: S1 S2 heard, no murmurs · RS: NVBS, no added sounds · P/A: soft, no organomegaly · CNS: NFND.”
- Blocks added before the eye:** neck (thyroid swelling moving with deglutition); local examination of the eye for an ulcer (lid closure, Bell’s phenomenon, lagophthalmos, trichiasis, entropion, ectropion, blepharitis, meibomitis, foreign body in the fornices); head posture and facial symmetry; cranial nerves I–XII for palsies.

Ocular examination — RE | LE table

Each row: right eye, then left eye. The RE column shows a normal finding as the department writes it; the LE column an abnormal example, mostly from the department’s or the model case sheets.

Row, in this order	Look for	RE — normal	LE — abnormal example
Visual acuity	Unaided → pinhole → best corrected (write the glass) → near	6/9, PH 6/6 6/9 C̄ +0.50 DSph → 6/6 N6 at 33 cm	6/60 NIGNIP
Head posture	Face turn, head tilt, chin position; facial asymmetry	Normal	Characteristic face turn to the left side
Hirschberg test and ocular movements	Corneal reflexes, before dilating; ductions and versions	Reflexes symmetrical (orthophoric); EOM full	Abduction restriction ⊕
Lids and adnexa	Position, oedema, closure, lagophthalmos, trichiasis; lacrimal sac regurgitation	Ⓝ	Lower-lid oedema ⊕; inferior scleral show ⊕
Conjunctiva	Congestion (circumcorneal or conjunctival), chemosis, bleb; evert the upper lid	Clear	Circumcorneal congestion ⊕
Cornea	Clarity; oedema, opacity, ulcer, vessels, KPs; sensation; staining	Clear; sensation intact	Greyish-white 4 × 4 mm paracentral ulcer, well-defined margins, surrounding stromal infiltrate
Anterior chamber	Central and peripheral depth (Van Herick); cells, flare, hypopyon	VH Gr IV (ND)	ND; hypopyon ___ mm, mobile / not mobile
Iris	Pattern; new vessels; atrophy; nodules; synechiae; PxF; iridotomy patency	CPN	PxF ⊕ at the pupillary margin
Pupil	Size, shape, direct and consensual; RAPD (swinging light) before dilating	RTL (3 mm)	4 mm, sluggish, grade 3 RAPD

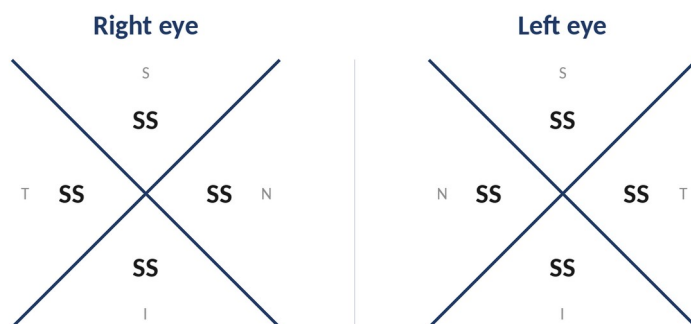
Row, in this order	Look for	RE — normal	LE — abnormal example
Lens	Cataract and grade, PCIOL, aphakia, subluxation; PxF	Clear	NS-3; PxF in ALC
Intraocular pressure	Method, value, time; before gonioscopy and dilatation	Tn by GAT 14 mm Hg at 10 am	Tn by GAT 26 mm Hg at 10 am
Gonioscopy	Deepest structure seen per quadrant, on a cross; PAS, new vessels	SS in all quadrants (open)	TM all round; patent ostium and iridectomy superiorly
Fundus	Distant direct → direct → +90 dioptre lens → indirect (card B)	Media clear; disc and vessels appear normal; macula FR ⊕	CDR 0.8, NRR thinning ⊕ (superior, inferior), bayoneting ⊕, PPA ⊕
Fields (confrontation)	Each quadrant against your own field	Full in all quadrants	Defect ⊕ in the ___ quadrant

Tension: before gonioscopy (which lowers the reading) and before dilating (which raises it); always the method and the time. The department's tables list "Tn by GAT" straight after visual acuity, which is fine on paper if you measured it first.

Van Herick: a thin slit 1 mm inside the temporal limbus at 60°; compare the peripheral chamber depth with the corneal thickness.

Grade 4 ≥ 1 (normal depth) · 3 ¼ – ½ · 2 ¼ · 1 < ¼ (potentially occludable). **No slit lamp — pen-torch method:** torch from the temporal side; whole iris lit = grade 4, normal depth; two-thirds lit = grade 2, shallow.

Gonioscopy: draw one cross per eye, write the deepest structure seen in each quadrant, and name the system ("by modified Shaffer's grading", as the department writes). Last year's exam sheet:



Deepest structure seen in each quadrant — scleral spur all round in both eyes: open angles (last year's exam sheet)

Fundus and fields: describe the fundus by card B; in a long case a normal fundus is written "Media clear; disc and vessels appear normal; macula FR ⊕". Confrontation: face to face, 60 cm apart; his right eye fixes your left eye, the other two eyes closed; bring your finger in from the periphery, midway between you, in each quadrant, and compare with your own field.

Summary

"Mr ___, a ___-year-old ___, came with ___ in the ___ eye for the past ___, ___ in onset, ___ in nature, associated with ___; no history of ___; known case of ___. On examination, visual acuity ___ / ___; ___ [positives]; ___ [key negatives]; intraocular pressure ___ / ___ mm Hg by ___ at ___." Only the positives that make the diagnosis and the negatives that exclude the differentials.

Complete diagnosis

Formula: eye → disease → type / stage / grade → aetiology → complications or status → other eye. Begin "My provisional diagnosis is ..."; say "probably of ... aetiology" when the cause is not proven.

Slot	Glaucoma sheet	Corneal ulcer sheet	Third nerve model sheet
Eye	Both eyes	Left eye	Right eye
Disease	primary open-angle glaucoma	corneal ulcer	third nerve palsy
Type / stage / grade	—	paracentral	pupil-sparing, infranuclear, incomplete
Aetiology	—	probably of bacterial aetiology	due to ischaemic microangiopathy
Complications or status	with pseudophakia, with intraocular pressure under control on medical management	in healing stage	—
Other eye	(both eyes)	state it if abnormal	state it if abnormal

More examples:

- "Bilateral axial proptosis, probably of thyroid-related orbitopathy, in active stage" (active or inactive by the signs).

- “Right eye: primary open-angle glaucoma, status post trabeculectomy with failed bleb and grade 2 nuclear sclerosis. Left eye: primary open-angle glaucoma and grade 2 nuclear sclerosis.”

Differentials, investigations and management

- **Differentials:** 2–4, most likely first, each with one point for and one against.
- **Investigations:** to confirm → to stage or grade → to find the cause → baseline before treatment. CCT goes here, not in the clinical table.
- **Management:** aim → medical (drug, strength, frequency, eye) → laser → surgery → follow-up and counselling.

Seniors’ corrections — do these every time

- **EOM** row straight after visual acuity.
- **Van Herick grade** for every anterior chamber (“VH Gr IV (ND)”); the seniors circled it.
- **CCT out of the clinical table:** they struck the row out; it is an investigation.
- **“Fields (confrontation test)”** after the fundus, before the diagnosis.
- **The drug and systemic checklist** in every case; **complete the negative:** “No h/o headache **with vomiting and pain**”.
- **Gonioscopy:** name the grading (“by modified Shaffer’s grading”) and draw the cross.
- **Corneal ulcer:** site, size, shape, depth, colour, margin, floor, vascularisation, KPs, stromal infiltrate; then sensation, staining, syringing, digital tension.
- **Cranial nerves:** name the muscles: the fifth nerve’s mandibular division supplies masseter, temporalis, medial and lateral pterygoids.

How to present it in 8–10 minutes

The older scheme gave the long case 20 + 10 minutes without saying how they are split. Prepare 8–10 minutes; if stopped, go straight to the summary and diagnosis (each card’s 2-minute say-it script).

Minute	Section	Key point
0–0.5	Particulars and chief complaints	One sentence: complaint, eye, duration
0.5–2.5	History of presenting illness	Onset, progression, associations; positives; negatives
2.5–3.5	Past, personal, family, treatment	Seniors’ checklist; drugs with strength and frequency
3.5–4	General and systemic examination	One sentence if normal; give the vitals
4–7.5	Ocular examination, RE then LE	Brisk on normal rows; slow on the key structure
7.5–8	Special tests for this case	Gonioscopy, Hertel, ptosis measurements, cranial nerves
8–8.5	Summary	Positives and key negatives
8.5–9	Complete diagnosis	“My provisional diagnosis is ...”, by the formula
9–10	Differentials, investigations, plan	One line each; then stop for questions

Model opening — the first 2 minutes

SAY IT

My patient is a 35-year-old man, a labourer by occupation, who came with complaints of pain, redness, watering and defective vision in the left eye for the past 10 days.

He was apparently normal 10 days back, when dust fell into his left eye while he was travelling on a two-wheeler. Then he had pain in that eye, which was sudden in onset, progressive in nature and associated with redness, with watering, photophobia and defective vision. He was treated at a private clinic nearby with two eye drops and then referred to our institute.

There is no history of contact lens wear, eye surgery, steroid drops or similar episodes in the past. He is not a known case of diabetes mellitus, hypertension, tuberculosis, asthma or epilepsy, and he has no drug allergy. His personal and family history are not significant.

Model closing — the last 2 minutes

SAY IT

To summarise, my patient is a 35-year-old labourer with pain, redness, watering, photophobia and defective vision in the left eye for 10 days after dust fell into it, with no contact lens wear, steroid use or diabetes. Visual acuity is 6/6 in the right eye and 6/60 in the left, not improving with glasses or pinhole. The left eye has lid oedema, circumcorneal congestion and a greyish-white, 4 by 4 millimetre paracentral corneal ulcer with well-defined margins and surrounding stromal infiltrate; the anterior chamber is of normal depth, with no hypopyon. The right eye and both fundi are normal.

My provisional diagnosis is left eye paracentral corneal ulcer, probably of bacterial aetiology, in healing stage. The main differential is a fungal ulcer, which would show feathery margins and satellite lesions.

I would take a scraping from the base and margins of the ulcer for Gram stain, a 10 per cent potassium hydroxide mount and culture, treat according to the result, with atropine for the ciliary spasm, and follow the ulcer until it heals.

Case-specific add-ons

What the typed formats ask for on top of this proforma:

Case	Extra examination	Cards
Glaucoma	Colour vision; bleb; Krukenberg spindle; new vessels, PxF, iridotomy patency; vertical and horizontal CDR, disc haemorrhage, nerve fibre layer defects; gonioscopy diagram; automated perimetry	G1-G10
Cornea	Lid apposition, punctum, tear meniscus; evert the upper lid; corneal sensation, staining; each slit-lamp illumination; ulcer site, size, shape, edge, margin; labelled colour diagram	C1-C11
Proptosis	Axial or eccentric; pulsation; change with Valsalva; thrill, finger insinuation, retropulsion; auscultation for bruit; Hertel exophthalmometry; pressure in upgaze; forced duction	O1
Paralytic squint	Head tilt, face turn; primary and secondary deviation; diplopia charting, Hess chart; Park-Bielschowsky three-step test; double Maddox rod; past pointing	N1-N3, O3
Strabismus	Alternate cover; Krimsky; prism bar cover test in nine gazes; A or V pattern; accommodative convergence/accommodation ratio; Worth four-dot, Bagolini, Titmus; Bruckner	Tools on N1-N3

Read more: Department case-sheet proforma (glaucoma, corneal ulcer, proptosis, uveitis, ptosis and sixth nerve sheets) · Typed case-presentation formats: glaucoma, cornea, proptosis, paralytic squint, strabismus · Aravind "Case sheet writing" p.649-667 · Baidya p.12-18, 149-150, 162 · Namrata p.177

B Master fundus format

Use with every fundus card · Fundus: 60 marks (2 × 30) · 10 + 5 minutes per case

The order

1. **Particulars and chief complaint** (shorthand as on card A), each eye with its own duration; history jotted beside it: diabetes, hypertension (duration, treatment), past surgery, injections or laser, night blindness, consanguinity.
2. **Vision with pinhole**, right eye (RE) above left eye (LE): “6/24 → PH 6/18”.
3. **Tonometry**: method, value, time (the department uses the non-contact tonometer).
4. **Anterior segment**: the four points below, before dilating.
5. **Fundus description**, aloud, in the order below.
6. **Drawing**, colour-coded, every lesion labelled.
7. **Diagnosis** with grade or stage and the lens status, one line per eye.
8. **Plan**: eye + treatment, and the investigation that decides it.

Anterior-segment points to check — before dilating

Point	Why	Record
New vessels on the iris (NVI)	Proliferative retinopathy, ischaemic vein occlusion: pupillary margin, high magnification	“NVI ⊕ at the pupillary margin”
Lens status	Part of the vision loss; a lens line in every diagnosis	“NS-2”, “PCIOL”
Pseudoexfoliation	Weak zonules, glaucoma; on the department’s vein occlusion sheet	“PxP in ALC”
Relative afferent pupillary defect (RAPD)	An ominous sign in vein occlusion; check before dilating	“Sluggish RTL”, “RAPD ⊕”

Fundus description — in this order

Step	Say it like this
Red glow	“On distant direct ophthalmoscopy at one arm’s distance, there is a good red glow.”
Media	“Direct ophthalmoscopy close to the face reveals clear media” (or “hazy media due to lens changes”).
Disc: size, shape, colour, margins, cup–disc ratio, rim	“The disc is normal in size, vertically oval, pink, with well-defined margins, a cup–disc ratio of 0.3 and a healthy neuroretinal rim.”
Vessels: origin, branching, arteriovenous (AV) ratio, calibre, crossings, sheathing	“The vessels arise from the centre of the disc, branching dichotomously, with an arteriovenous ratio of 2:3” (after the first branching); then attenuation, beading, AV crossings, sheathing.
Background	Each lesion: number, colour, shape, size in disc diameters, margins, location → “suggestive of ...” (table below).
Macula	Foveal reflex present or absent; oedema; hard exudates (circinate pattern?); scar.
Periphery	“Indirect ophthalmoscopy with a 20 dioptre lens shows the peripheries to be normal.”
Confirm	“On slit-lamp biomicroscopy with a plus 90 dioptre lens, the above findings were confirmed.”

Describe each lesion, then name it

Lesion	Say it like this
Hard exudates	“Yellowish, waxy lesions with distinct margins around the fovea, suggestive of hard exudates.”
Dot and blot haemorrhages	“Red, pinhead-shaped lesions in all four quadrants, suggestive of dot and blot haemorrhages.”
Cotton-wool spot	“A yellowish, fluffy lesion with indistinct margins, suggestive of a cotton-wool spot.”
New vessels on the disc	“A fine, lacy frond of vessels on the disc, suggestive of neovascularisation of the disc.”
New vessels elsewhere	“A whitish fibrous band with tufts of vessels, 1 disc diameter inferonasal to the disc, suggestive of neovascularisation elsewhere.”
Old laser marks	“Atrophic, hypopigmented chorioretinal scars along the arcade, suggestive of old laser marks.”

Say it — a normal fundus

SAY IT

Fundus examination of the right eye. On distant direct ophthalmoscopy at one arm's distance, there is a good red glow. Direct ophthalmoscopy close to the face reveals clear media. The disc is normal in size, vertically oval and pink, with well-defined margins, a cup-disc ratio of 0.3 and a healthy neuroretinal rim. The vessels arise from the centre of the disc and branch dichotomously, maintaining an arteriovenous ratio of 2 is to 3, with no attenuation, crossing changes or sheathing. The background retina is normal, with no haemorrhages, exudates or pigment. The macula is normal, and the foveal reflex is present. On indirect ophthalmoscopy with a 20 dioptre lens, the peripheries are normal. On slit-lamp biomicroscopy with a plus 90 dioptre lens, the above findings were confirmed. The left fundus is the same.

Draw it

Two circles side by side, the right eye on the left: write "RE" and "LE" above them (the department's circles are never labelled). Disc on the nasal side, cup-disc ratio beside it; every lesion gets a label and a pointer.

Colour	Standard code — draw this	What your department's sheets actually use
Red	Flat (attached) retina and haemorrhages	Haemorrhages of every kind; new vessels; one writer's fundus outline
Blue	Detached retina; retinal vessels (usually veins)	Disc outline; vessels, arteries and veins alike; labels and pointers
Red with a blue outline	Retinal breaks	Not used (no detachment last year)
Black	Pigment	Bony spicules, macular pigment, laser marks
Yellow	Exudates	Hard exudates; also disciform scar, sclerosed vessels, coloboma, macular flecks
Green	Vitreous opacities	Vitreous haemorrhage, fibrovascular proliferation, epiretinal membrane
Orange	Not in the code	Drusen

Your books add: arteries red, veins blue; drusen and coloboma yellow; sclerosed vessels black; silicone oil green; choroidal lesions brown. Where the department differs, use the standard colour and label it.

Diagnosis and plan

- **"Diagnosis:"**, one line per eye and condition, with the lens status and the grade the sheets leave out: "Right eye high-risk proliferative diabetic retinopathy with clinically significant macular oedema · right eye pseudophakia".
- **Plan**, eye + treatment: "(BE) panretinal photocoagulation (PRP)", "LE sectoral PRP", "LE intravitreal bevacizumab, 3 doses" (written "Inj Avastin").
- **Investigations:** macular optical coherence tomography, fundus fluorescein angiography, B-scan if the media are hazy.

Common fundus-description mistakes

- Naming a lesion without describing it (colour, shape, size, margins, number, location, then "suggestive of").
- Skipping the red glow and the media; a cup-disc ratio without the rim; no AV ratio.
- "Macula normal" without the foveal reflex; no periphery; no 90 dioptre confirmation.
- Pupil and iris checked after dilating, so an RAPD or iris new vessels are missed.
- A diagnosis without its grade, stage or lens status; unlabelled circles, eyes swapped; abbreviations aloud ("NVE" for new vessels elsewhere).

Read more: *Department fundus case sheets (13 cases)* · *Department fundus write-ups (case-sheet proforma p.21, 30)* · Aravind "Case sheet writing" p.649–650; 1.7; 6.8 · Baidya p.748–749 · Namrata p.196, 218 · Kanski p.27

C Master short-case method

Use with every short-case card · Short cases: 60 marks (2 × 30) · 10 + 5 minutes each

The method, in order

1. **Hear the instruction** and do exactly that ("Examine the anterior segment of this patient").
2. **Introduce yourself and take consent**; ask the age and complaint only if the examiner allows.
3. **Inspect before you touch**: head posture, face, both eyes compared, lids, deviation.
4. **Focused examination**, torch then slit lamp, so the examiner sees each sign: pupils (relative afferent pupillary defect) and iris new vessels before dilating; tension before gonioscopy.
5. **Describe aloud**, right eye then left (always the other eye too): each finding described, then "suggestive of ..."; no abbreviations.
6. **Complete diagnosis line**: eye → disease → type / stage / grade → aetiology → complications or status → other eye.
7. **Differentials**: 2–3, each with one distinguishing point.
8. **Investigations**: the one or two that decide.
9. **Management**: one line.

Timing: minutes 0–1 instruction, consent, inspection · 1–7 examination · 7–10 description to management · then 5 minutes of questions. The split is not official: if asked for the diagnosis early, give it at once.

Glaucoma short cases were do-and-show

Last year most of the 12 glaucoma patients carried a task to perform in front of the examiner:

Task	Last year	Do it, then record
Applanation tonometry (card G8)	8 of 12	Consent, instruction, topical anaesthetic, fluorescein, disinfected prism; both eyes; eye, method, value, time
Gonioscopy (card G9)	8 of 12, every angle-closure glaucoma	After tonometry, before dilating; anaesthetic, disinfected lens; one cross per eye, deepest structure per quadrant; name the grading
Humphrey field (card G10)	3 of 12	Reliability first; defect on the pattern deviation plot; stage; match it to the disc

Treatment status was noted ("on anti-glaucoma medication" or not), so always state the current drops. Be ready for all three tasks on any glaucoma patient.

Say it — a short-case model

Instruction: "Examine the anterior segment of this patient and measure the intraocular pressure."

SAY IT

On slit-lamp examination of the right eye, the lids and conjunctiva are normal and the cornea is clear. The anterior chamber is of normal depth, Van Herick grade 4. There is white, flaky, dandruff-like material at the pupillary margin as tufts and on the anterior lens capsule as a disc-shaped deposit, with no iris atrophy, suggestive of pseudoexfoliation. The pupil is round and reacting to light, and the lens shows grade 2 nuclear sclerosis. The left anterior segment is normal. Intraocular pressure by Goldmann applanation tonometry is 24 millimetres of mercury in the right eye and 16 in the left, at eleven in the morning.

My diagnosis is right eye pseudoexfoliation syndrome with raised intraocular pressure and grade 2 nuclear sclerosis. The differentials are pigment dispersion syndrome, with mid-peripheral iris transillumination defects and a Krukenberg spindle, and true exfoliation, a delamination of the lens capsule after heat or infrared exposure.

I would do gonioscopy and a dilated disc examination to decide whether this is pseudoexfoliation glaucoma, with a Humphrey field and central corneal thickness. If it is, I would start medical treatment; at cataract surgery I would plan for a poorly dilating pupil and weak zonules.

Read more: Last year's glaucoma case list (tasks noted) · Cards G6, G8, G9, G10 · Baidya p.150, 155, 168–169 · Namrata p.177, 204 · Aravind 4.11; "Case sheet writing" p.666

D Card index

★ = kept at JEH last year · 61 cards in 7 files · 21 cards marked ★

Every card, by file

01_Glaucoma_Case_Cards

ID	Card	Type	★ last year
G1	Primary open-angle glaucoma (POAG), including the glaucomatous optic disc	L+S	★ ×6
G2	POAG with trabeculectomy: bleb assessment, failed bleb	L	
G3	POAG with a glaucoma drainage device	L	
G4	Primary angle-closure disease, including primary angle-closure glaucoma (PACG) and after laser iridotomy	L	★ ×4 (1 long case)
G5	Neovascular glaucoma, including rubeosis iridis	L	★ ×1
G6	Pseudoexfoliation syndrome and glaucoma	L+S	★ ×1 (one of G1's 6)
G7	Central retinal vein occlusion (CRVO) with POAG; hemi-CRVO with POAG	S	★ ×2
G8	Applanation tonometry: perform and record	T	★ task in 8
G9	Gonioscopy: perform, grade, draw	T	★ task in 8
G10	Humphrey field in glaucoma	C	★ task in 3

02_Retina_Fundus_Case_Cards

ID	Card	Type	★ last year
R0	Normal fundus: the perfect description	S	
R1	Proliferative diabetic retinopathy, including laser marks and scars	L+S	★ ×3
R2	Non-proliferative diabetic retinopathy with diabetic macular oedema	L+S	
R3	CRVO: ischaemic, non-ischaemic, hemi-CRVO	L+S	★ ×2 (as G7)
R4	Branch retinal vein occlusion, including superotemporal	L+S	★ ×2
R5	Age-related macular degeneration, wet and dry	L+S	★ ×2
R6	Retinitis pigmentosa	L+S	★ ×4
R7	Retinal detachment	L	
R8	Hypertensive retinopathy	S	
R9	Stargardt disease	S	★ ×1
R10	Macular hole	S	
R11	Coloboma: iris, chorioretinal, disc	S	★ ×2
R12	Silicone-oil-filled eye	S	
R13	Fundus fluorescein angiography: diabetic retinopathy, new vessels, non-perfusion	C	
R14	Diabetic retinopathy chart: grading	C	
R15	Retinopathy of prematurity chart	C	
R16	Fundus photographs: spotters	C	

03_Cornea_Case_Cards

ID	Card	Type	★ last year
C1	Fungal corneal ulcer	L+S	★ ×2 (1 long case)
C2	Bacterial corneal ulcer, including hypopyon ulcer	L	
C3	Failed graft and the post-keratoplasty eye	L+S	★ ×1
C4	Keratoconus	L	
C5	Pseudophakic and aphakic bullous keratopathy	L	

ID	Card	Type	★ last year
C6	Opaque cornea, including adherent leucoma	L+S	
C7	Nummular keratitis	S	
C8	Corneal dystrophy	S	
C9	Corneal degeneration	S	
C10	Band-shaped keratopathy	S	
C11	Atheromatous ulcer	S	

04_Lens_Uvea_Misc_Case_Cards

ID	Card	Type	★ last year
M1	Ectopia lentis; Marfan syndrome with subluxated lens	L+S	★ ×1
M2	Dislocated lens in the vitreous	L+S	★ ×1 (fundus list)
M3	Anterior subluxation or dislocation of the lens	L+S	
M4	Dislocated or subluxated intraocular lens	L+S	
M5	Developmental (zonular, lamellar) cataract and aphakia	S	★ ×1
M6	Traumatic cataract	S	
M7	Uveitis: festooned pupil, iris nodules, synechiae	L+S	
M8	Hyphaema	S	
M9	Hypopyon and inverse hypopyon	S	

05_Nerves_Case_Cards

ID	Card	Type	★ last year
N1	Third nerve palsy	L+S	★ ×1
N2	Fourth nerve palsy	L+S	
N3	Sixth nerve palsy	L+S	
N4	Optic atrophy	L+S	
N5	Anterior ischaemic optic neuropathy, non-arteritic and arteritic	L+S	
N6	Facial nerve palsy, including lagophthalmos	L+S	

06_Oculoplasty_Case_Cards

ID	Card	Type	★ last year
O1	Proptosis, including thyroid eye disease	L+S	
O2	Ptosis	L+S	
O3	Ocular myasthenia gravis	L+S	
O4	Anophthalmic socket	L+S	
O5	Contracted socket	L+S	

07_Viva_Charts_Instruments

ID	Card	Type	★ last year
V1	Visual fields: glaucoma and neurological	C	
V2	Imaging: optical coherence tomography, angiography, B-scan, corneal tomography, specular microscopy, ultrasound biomicroscopy, electrophysiology, computed tomography and magnetic resonance imaging	C	
V3	Instruments, by surgery	C	

Key: ★ kept at Joseph Eye Hospital (JEH) last year; ×n = how many of last year's 34 patients had it (for a task, how many of the 12 glaucoma patients carried it) • L long case • S short case • L+S one card with both versions • T task you perform • C chart or picture. Counts overlap: G6's patient is one of G1's six, and R3 counts the same two patients as G7.

Marks and timings

Head	Marks	Time for drills
Long case	80 (1 × 80)	20 + 10 min
Short cases	60 (2 × 30)	10 + 5 min each
Fundus	60 (2 × 30)	10 + 5 min (one case in the older scheme)
Refraction	30 (1 × 30)	10 + 5 min
Ward rounds	30 (5 × 6)	3 + 2 min per patient
Objective structured clinical examination	20 (5 × 4)	2 min per station
Dissertation	20	
Clinical total	300	
Viva, including competency	80 (60 + 20)	
Log book	20	
Aggregate	400; pass 200	

- **Marks:** the university's postgraduate practical scheme, as modified at the 64th Standing Academic Board on 25 November 2025.
- **Times:** from the older 300-mark scheme (last year's photo 1). Its marks no longer apply, but its timings are the only official ones found, so use them to time practice.

Last year's 34 cases at JEH

Category	Diagnoses (count)	Notes
Glaucoma (12)	POAG ×6 (one with pseudoexfoliation glaucoma; one both eyes) · PACG ×4 · neovascular glaucoma ×1 · CRVO with POAG ×1	Tasks written: applanation tonometry 8, gonioscopy 8, Humphrey field 3. Gonioscopy drawn as a cross: scleral spur in all four quadrants, both eyes.
Fundus (16)	Retinitis pigmentosa ×4 · proliferative diabetic retinopathy ×3 · age-related macular degeneration ×2 · superotemporal branch retinal vein occlusion ×2 · coloboma ×2 · hemi-CRVO with POAG ×1 · Stargardt disease (both eyes) ×1 · dislocated lens in the vitreous ×1	The hemi-CRVO patient was marked both fundus and glaucoma.
Anterior segment (5)	Fungal corneal ulcer ×2 · failed graft ×1 · subluxated lens (Marfan syndrome) ×1 · lamellar cataract / aphakia ×1	
Cranial nerve (1)	Third nerve palsy	On the main list only.
Total	34	

- **The iris coloboma:** the main list marks one coloboma patient as anterior segment ("iris coloboma"), while the category sheets list her under fundus. By the main list, fundus is 15 and anterior segment 6.
- **Two case-sheet long cases:** one PACG and one fungal corneal ulcer were taken as full written case sheets. Practise writing a complete case sheet (card A) for both.

Read more: Department long- and short-case checklist · Last year's case lists (photos 2–7) · Older practical scheme (photo 1) · Tamil Nadu Dr M.G.R. Medical University postgraduate practical scheme, 64th Standing Academic Board, 25 November 2025